

Colpocleisis (Vaginal Closure Surgery)

Patient Information · A prolapse repair that closes the vagina

WARWIKI

Colpocleisis is a surgery for pelvic organ prolapse. It **closes most of the vagina** to hold the organs up. Because it closes the vagina, it is only for people who are sure they no longer want vaginal intercourse. It is often chosen by people whose health makes a bigger repair riskier.

About This Procedure

Most prolapse surgery rebuilds the support of the vagina. Colpocleisis works a different way. The surgeon **removes strips of the vaginal lining and stitches the walls together**. This makes the vagina short and closed, so the organs cannot drop out.

It can be done with the uterus left in place (a LeFort repair) or after a past hysterectomy. Removing the uterus at the same time is not usually needed.

Benefits and Risks in Numbers

95 in 100

people were satisfied with their choice a year later

1 in 7

have bothersome leaking with coughing or effort a year later

8 in 100

need another prolapse operation within 10 years

Numbers are averages from published studies. Your own risk may differ; ask your team.

The Key Decision

After colpocleisis, **vaginal intercourse is no longer possible**. This cannot be undone. Touching and other kinds of sexual activity, including stimulation of the clitoris, are still possible. Think about whether your wishes could change in the future. Age alone is not a reason to choose this surgery.

Learn the Terms

Prolapse

When pelvic organs drop and cause a bulge or pressure.

Colpocleisis

Surgery that closes most of the vagina to hold the organs up.

Obliterative surgery

A repair that closes the vagina instead of rebuilding it.

LeFort

A type of colpocleisis done with the uterus left in place.

Sling

A support that may be added for leaking with coughing or effort.

Catheter

A thin tube that drains urine from the bladder.

Will it hurt? You will be asleep or numb during surgery, so you will not feel it. Afterward, most people have mild soreness and some spotting. Pain pills and soft stools help.

First day

Walk soon after surgery. Your bladder emptying is checked before or after you go home.

First weeks

Spotting and discharge are normal. Keep stools soft. Do everyday activities as you feel able.

Follow-up visits

Your team checks healing. A later visit, often around 6 months, includes an exam.

How to Get Ready

- Be **sure about the decision** not to have vaginal intercourse.
- Tell your team about any **bleeding from the vagina**. The uterus and cervix are harder to check after surgery, so they are checked first.
- Ask whether a **sling** makes sense if you leak urine. It can be done now or later.
- Follow the instructions about eating, drinking and blood thinners. Arrange a ride home.

What Happens During Surgery

- 1 You get general or spinal anesthesia, and antibiotics.
- 2 Working through the vagina, the surgeon removes strips of lining and **stitches the walls together**.
- 3 Your bladder is checked. A catheter may be placed for a short time.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Bleeding that is more than light spotting, or discharge that smells bad
- Pain that is getting worse, or new bleeding from the vagina later on

Go to an emergency department if:

- You **cannot urinate** for several hours and your bladder feels full or painful
- Bleeding that soaks a pad in an hour
- You cannot reach the office and are worried

Three Things to Remember

1. Colpocleisis closes most of the vagina. It relieves prolapse for most people and is often chosen when a bigger repair would be riskier.
2. The change is permanent: vaginal intercourse is no longer possible.
3. Any new bleeding from the vagina later in life still needs to be checked.